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Name: سعيدة محمد احمد ابراهيم National ID: 26807022702369	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

معيده

محمد احمد ابراهيم

١٨ ش زمزم-الهجانه
مدينه نصر اول - القاهره

١٩٦٨/٠٧



٢٦٨٠٧٠٢٢٧٠٢٣٦٩

LB0618362



٢٠٢٥/٠٣ ٢٦٨٠٧٠٢٢٧٠٢٣٦٩

بدون عمل



أنثى مسلمة متزوجة
عبدالكريم محمود منصور ابراهيم
البطاقة سارية حتى ٢٠٣٢/٠٣/٢٦



Dms
← كيمافى
← تدعيم



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
طبية
مصر

06/14/2026

MDT FORM

Name: سعدة محمد احمد إبراهيم	MRN: 10294
Age: 58 Y	ID: 26807022702369
Menopausal Sta: post	Phone: 01121056208
NRS SCORE:	Marital Status: married
BMI:	Family History: -ve
CO Morbidity: HTN	Reason Of MDT: New

Clinical Data:

BX: Rt IDC 2
ER: 8/8 PR: 8/8 HER2: 1+ KI67: 20 %

Sonomammography:

Rt UOQ multifocal masses + overlying skin involvement + intraductal extension

Mets Workup:

CT CAP .. lingular atelectic plates with traction bronchiatasis + moasic attenuation of both lung fields

History:

T 4b N 1 M 0

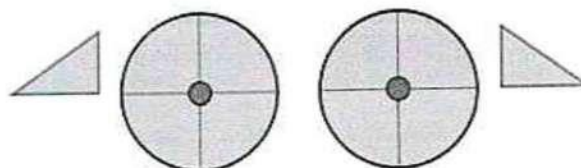
Recommendations:

MDT Outcome & Plan

for neoadj AC 4 TAXOTERE 4 for MRM ALND

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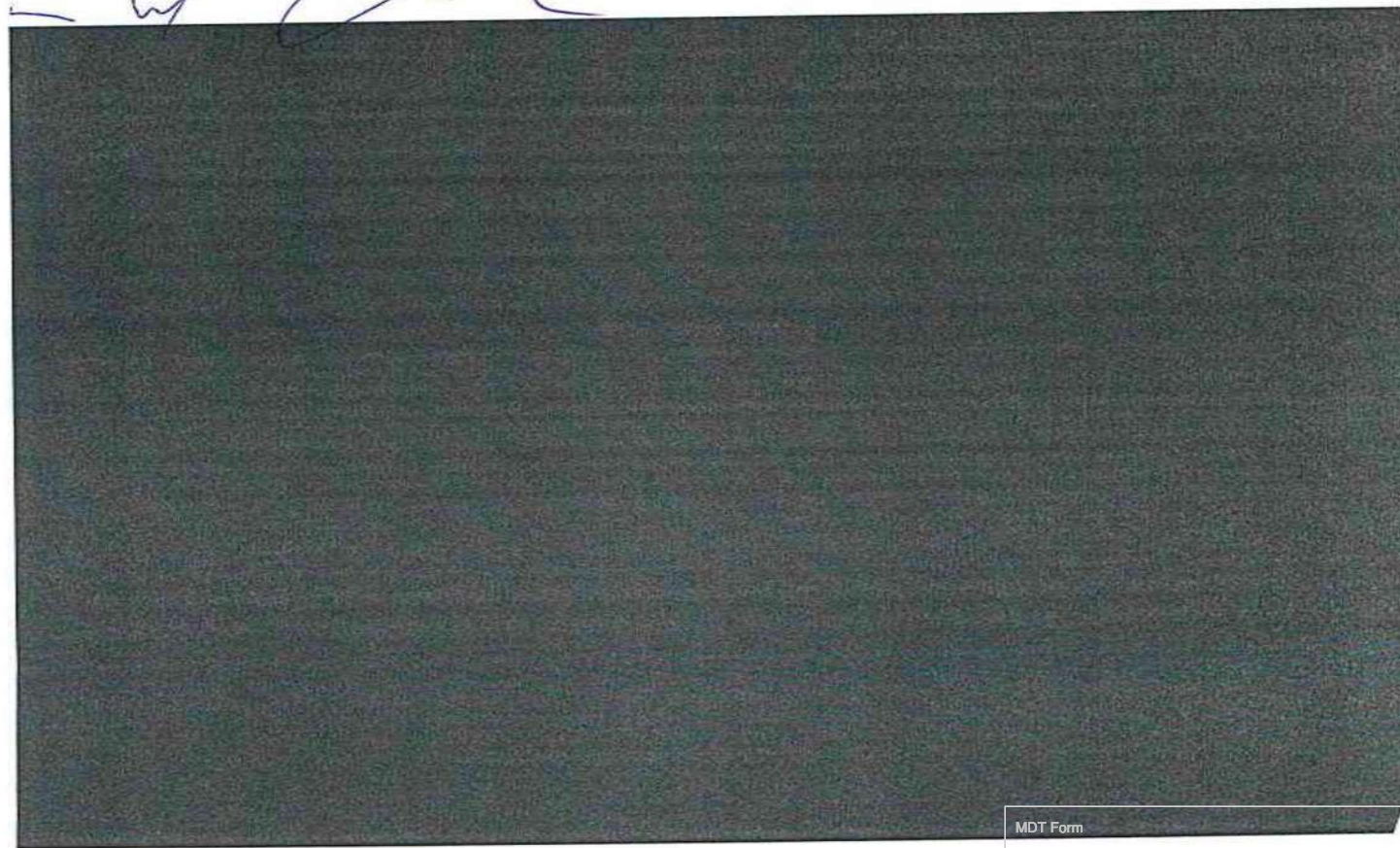
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TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr Hanan Ezzat	Dr Ahmed naguib				



MDT Form

[[DOCTYPE:MDT_FORM]]

Name: سعيد محمد احمد ابراهيم		
MR. : 10294		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه POST MENO right breast IDC G2 ER 8/8 PR 8/8 HER2 1 Rt UOQ multifocal masses + overlying skin involvement + intraductal extension CT CAP .. lingular atelectic plates with traction bronchatasis + moasic attenuation of both lung fields FOR C1 AC add controloc and ondalenz labs pending clinical pharmacy note : 4/7/2026 ef 60% wt 95 ht 154 sa2 for adria 120 endoxan 1200 for c2 adria 120 endoxan 1200 + supp ttt after labs postm rt breast cancer ct4b n2 mo idc 2 er8/8 pr8/8 her2-ve ki 20% cts chronic small airway disease bs pending for c4 ac adria 120 endoxan 1200 and dexta prep on exam rt retroaleolar mass 3x4 right ulcerative lesion healed rt ax ln 1x2. The patient has completed 4 cycles of AC adria 120 endoxan 1200.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
	شريف الشناوى	
يعتمد ,		

Women's Health Outreach Program

Name: saeida Mohamed

Date: 31-3-2026

Bilateral digital mammography and high resolution ultrasonography examination

Clinical Summary: right breast lump one month duration in 57 years old patient .

Detailed Findings

Right breast

- Scattered fibroglandular mammary parenchyma ACR b.
- Right breast UOQ ,is the seat of multiple ill defined speculated dense mass with suspicious nature
- **Complementary us :** prominent fibroglandular echopattern with mottling appearance .
- An irregular shaped heterogeneous echopattern multiple solid mass lesions occupying from 9 o'clock till 12-1 o'clock region with internal vascularity by color Doppler study it measures as follows :
 - At 9 o'clock measures 16x14mm
 - At 10-11 o'clock measures 20x15mm with this lesion seen connected to overlying skin
 - At 12-12 o'clock measures 19x20mm
 - At 12-1 o'clock measures 7x5mm .
- Multiple Dilated ducts reaching size of 2.4mm some show internal contents
- two pathological axillary LNS level I with cortical thickening reaches 3mm .

left breast

- scattered fibroglandular mammary parenchyma ACR b.
- **Complementary us**
 - prominent fibroglandular echopattern with mottling appearance .
 - No solid or cystic lesion detected .
 - Normal skin/nipple/areolar complex.
 - Non-specific axillary LNs.

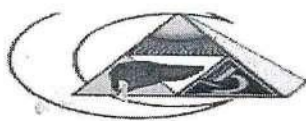
Conclusion:

- **Right breast: (BI-RADS 5)** UOQ highly suspicious multifocal neoplastic process P With ipsilateral pathological lymph nodes , overlying skin involvement and suspected intraductal extension .
- **Left breast: (BI-RADS 2)** Fibroadenosis . .

For Tru cut biopsy and contrast mammography are recommended.

Much Obligated,

DR/marwa a. el seoud



NHTMRI

قسم الباثولوجي

الهيئة العامة للمستشفيات والمعاهد التعليمية
المعهد القومي للكبد والجهاز الهضمي
والأمراض المعدية

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: سعيده محمد احمد ابراهيم	Age: 58years	Sex: female
Request type: مبادره	Sub request type:	
Hospital no:	Pathology no.:184B-2026	
Department: صحه مرأه	Date of result:7/4/2026	
Referred by: د/امل حجازي	Date of biopsy:1/4/2026	
Nature of specimen: tru-cut biopsy	I.D. Number:26807022702369	

Clinical diagnosis: Right breast mass

Gross pathology :

Multiple fragmented grayish white tissue cores up to 1cm long, totally submitted.

Immunohistochemical Technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain. The sections were treated against Ecadherin-ER- PR- Her2- KI67

Microscopic picture: Section examined revealed core of mammary tissue showing invasive duct carcinoma of no special type (NST). The tumor forms nest, cords, and cribriform pattern of anaplastic ductal cells infiltrating among desmoplastic stroma. Tumor cells showing mderate anaplasia and moderate tendency to from tubules. There is mild lymphocytic response. No peritumoral lymphovascular emboli or peri neural infiltration in core tissue examined. Also seen ductal carcinoma insitu component, solid subtype of mild to intermediate grade

Immunohistochemical Result

ER: Positive Strong nuclear staining in 80% of tumor cells (Allred Score 8/8).

PR: Positive Strong nuclear staining in 80% of tumor cells (Allred Score 8/8).

Her2:Weak incompelete membranous staining of >10% tumor cells (Score 1+) Her2 low.

Ki67: Positive nuclear staining of 20% of tumor cells.

Diagnosis: Right breast mass, tru-cut biopsy:

Invasive Ductal carcinoma (NST), Grade II.

Signature:

Dr.Mona Mohamed.M.D

Pathology Lab. Manager

Nermine Nabil, M.D

Patient's Name: Saeeda Mohammed Ahmed Ibrahim
Date: 2026-05-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

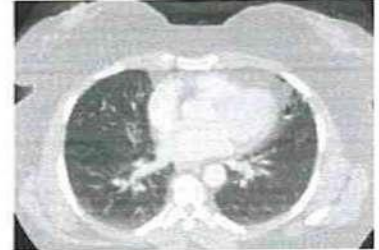
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**POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS
WITH CORONAL RECONSTRUCTION: -**

I. Chest

• **Lungs:**

- Lingular atelectatic plates with traction bronchiectasis in keeping with old inflammatory insult.
- Mosaic attenuation of both lung fields with geographical areas of air trapping.



- **Pleura:** No pleural effusion. No pleural-based mass or thickening.
- **Mediastinum and Hila:** No pathologically enlarged mediastinal or hilar lymph nodes identified.
- **Cardiomediastinal contours:** Cardiac size within expected limits for technique. No acute cardiopulmonary process identified.

II. Abdomen

- **Liver:** Normal size and contour for technique. Homogeneous parenchymal attenuation without focal hepatic mass identified. No intrahepatic biliary dilatation.
- **Gallbladder and Biliary Tree:** Gallbladder unremarkable; no gallstones or wall thickening identified. Common bile duct not dilated.
- **Pancreas:** Unremarkable in size and attenuation; no focal mass identified.
- **Spleen:** Normal size and attenuation; no focal lesion.
- **Adrenals:** Both adrenal glands demonstrate normal morphology without discrete mass or suspicious enlargement.
- **Kidneys and Collecting Systems:** Both kidneys demonstrate normal size, contour, and enhancement. No focal renal mass identified. No hydronephrosis. No suspicious perinephric stranding.
- **Vasculature:** Abdominal aorta and visualized major branches without acute aneurysm or dissection on the images obtained.

III. Pelvis

- **Bowel and Mesentery:** No focal bowel wall thickening or obstructive pattern identified. No suspicious mesenteric mass or pathologic lymphadenopathy.
- **Pelvic Organs:** Visualized portions of pelvic organs unremarkable for the study; no discrete adnexal or pelvic mass identified. IUD is seen in place.
- **Bladder:** Partially distended bladder without intraluminal mass identified.

Patient's Name: Saeeda Mohammed Ahmed Ibrahim
Date: 2026-05-12
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

IV. Osseous Structures

- Grade 1 degenerative spondylolisthesis of L4 & L5 vertebrae.

V. Additional

- No free intraperitoneal air or free fluid identified.

Impression:

- Lingular atelectatic plates with traction bronchiectasis in keeping with old inflammatory insult.
- Mosaic attenuation of both lung fields... chronic small airway disease.

Yours,
Dr. Yahya Khalifa ,Msc, FEBR



جمهورية مصر العربية
بطاقة تحقيق الشخصية

معيده

محمد احمد ابراهيم

١٨ ش زمزم-الهجانه
مدينة نصر اول - القاهرة

١٩٦٨/٠٧

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بدون عمل



أنثى مسلمة متزوجة
عبد الكريم محمود منصور ابراهيم
البطاقة سارية حتى ٢٠٣٢/٠٣/٢٦

